Australia's Public Hospital Wait Time Crisis: A Comparative Analysis of Victoria and New South Wales

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Prepared as part of an independent BA portfolio project

1. Executive Summary

This report examines the causes of declining Emergency Department (ED) performance across Victoria and New South Wales (NSW), using real data from the Australian Institute of Health and Welfare (AIHW) and the Australian Medical Association (AMA). Analysis spanning 2011-12 to 2024-25 reveals a sustained and worsening crisis that predates COVID-19 and has continued despite increasing government funding. Victoria's 4-hour rule performance dropped from 74% to just 56% over this period, while NSW declined from 88% to 74%. No state met the national 72% benchmark in 2022-23.
Six interconnected root causes were identified: population demand, critically low bed capacity, chronic workforce understaffing, access block, inefficient patient flow, and primary care policy gaps. In response, six prioritised recommendations are proposed covering pre-ED triage, bed capacity expansion, workforce recruitment, staff training, an escalation protocol for bed sourcing, and expanded after-hours GP services. Effective implementation is projected to reduce unnecessary ED presentations, eliminate access block, and deliver sustained performance improvement.

2. Background

Australia's public hospital system is facing a prolonged and worsening performance crisis that predates the COVID-19 pandemic. Despite gradual increases in government funding, key performance indicators have declined consistently across emergency department wait times and hospital capacity over the past decade.
The hospital system is experiencing what the Australian Medical Association (AMA) describes as "access block," a situation where patients cannot move through the system because capacity is unavailable downstream, creating a logjam that begins in emergency departments and flows throughout the entire system. This case study analyses that crisis through the lens of Australia's two most populous states: Victoria and New South Wales (NSW).

3. The National Problem

Australia's hospital bed capacity has failed to keep pace with population growth. Between 2017-18 and 2021-22, only 1,220 new public hospital beds became available nationally. This means Australia installed only 11.6 new beds for every 10,000 new Australians, less than half the existing ratio of 25.3 beds per 10,000 Australians as of 2017-18. (AMA 2024, p.5)

The problem is compounded by an aging population. Australians over 65 represented 17.1% of the total population in 2022, yet accounted for over 40% of total hospital separations and 47% of total patient days in 2021-22. (AMA 2024, p.6) Meanwhile, the number of beds available per 1,000 Australians aged over 65 has fallen to just 14.3, the lowest on record, down from 32.5 beds in 1991-92. (AMA 2024, p.6)

As a direct consequence of this capacity squeeze, fewer emergency patients are being seen on time. The 2022-23 national average of patients seen within clinically recommended timeframes was the

lowest in the past ten years across all four triage categories: Emergency (68%), Urgent (58%), Semi-Urgent (68%), and Non-Urgent (88%). (AMA 2024, p.9) The proportion of all Emergency Department (ED) visits completed within four hours fell to just 56% in 2022-23, down from 71% in 2017-18 and the lowest figure since 2011. (AMA 2024, p.8)

Importantly, the AMA notes clearly that this decline began well before COVID-19 and cannot be attributed to the pandemic alone. (AMA 2024, p.7)

4. The Expenditure Paradox

Between 2011-12 and 2021-22, Commonwealth Government hospital funding grew at an average annual rate of 2.23% per person, while state and territory funding grew at 2.47% per person. (AMA 2024, p.11) Despite this sustained increase in investment from both levels of government, performance has declined across every major metric. This finding is central to this analysis: it suggests that financial investment alone is insufficient, and that systemic process-level failures are a primary driver of poor outcomes.

5. State Analysis: Victoria and New South Wales

5.1 Victoria

Victoria presents a mixed but concerning picture. While the state has seen significant funding growth, with the Victorian Government increasing hospital spending at an average annual rate of 8.36% per person between 2016-17 and 2021-22, performance outcomes have deteriorated. (AMA 2024, p.20)

In 2022-23, the percentage of Victorian ED patients completing their visit within four hours fell to just 52%, compared to 71% only six years prior. (AMA 2024, p.18) The proportion of Category 2 planned surgery patients admitted within the recommended 90-day timeframe stands at just 55%, among the worst in Australia. (AMA 2024, p.19) Victoria's performance fell across most key indicators between 2021-22 and 2022-23. (AMA 2024, p.17)

5.2 New South Wales

NSW presents an equally troubling but different profile. Despite being the overall best-performing state nationally across several metrics, NSW was one of only two states to see performance decline across all four key indicators in 2022-23. (AMA 2024, p.13)

The proportion of NSW patients completing their ED visit within four hours fell from 75% in 2016-17 to just 59% in 2022-23. (AMA 2024, p.14) NSW reclaimed the unwanted title of longest median surgery wait time in the country in 2022-23, with patients now waiting more than twice as long as they did twenty years ago. (AMA 2024, p.15) The proportion of Category 2 surgery patients

admitted within the recommended timeframe dropped from over 95% pre-COVID to under 75% in 2022-23. (AMA 2024, p.15)

The AMA NSW President noted that the health system "is crumbling under unprecedented demand, while staff try to manage with fewer health dollars." (AMA 2024, p.13)

6. Stakeholder Analysis

6.1 Stakeholder Register

The stakeholder register below identifies all parties with an interest in or influence over the ED performance crisis across Victoria and NSW. Stakeholders are classified as internal or external, assessed against their level of power and interest, and assigned an engagement strategy accordingly.

Stakeholder	Type	Interest in this issue	Power	Interest	Engagement Strategy
Patients (general)	External	Timely, safe care. Directly harmed by long wait times and Did Not Wait (DNW) events	Low	High	Keep Informed
Emergency patients	External	Immediate treatment within clinically recommended timeframes. Highest personal risk if not seen on time	Low	High	Keep Informed
Elderly patients (65+)	External	Disproportionate users of the system — 47% of patient days despite being 17% of population. Most vulnerable to delays	Low	High	Keep Informed
Doctors (GPs and specialists)	Internal	Safe working conditions, manageable patient loads, adequate resources. Currently described by AMA as "exhausted due to chronic understaffing"	Medium	High	Manage Closely
Nurses and clinical staff	Internal	Workload management, patient safety, staffing ratios	Medium	High	Manage Closely
Hospital administrators / executives	Internal	Operational efficiency, budget management, performance targets, reputation	High	High	Manage Closely
Victorian Department of Health	External	State-level performance outcomes, policy compliance, public accountability	High	High	Manage Closely
NSW Ministry of Health	External	State-level performance outcomes — NSW declining across all four key metrics in 2022-23	High	High	Manage Closely
Commonwealth Government	External	National funding agreements, National Health Reform Agreement (NHRA), federal budget	High	Medium	Keep Satisfied
Australian Medical Association (AMA)	External	Advocacy for doctors and patients. Publishes annual report card to hold governments accountable	Medium	High	Keep Informed
Ambulance services	External	Directly impacted by ED access block — ambulances are ramped outside hospitals when EDs are full, delaying responses to new emergencies	Medium	High	Manage Closely
Private hospitals	External	Benefit when public system is under pressure as patients seek alternatives. Relevant to overall system capacity	Medium	Low	Monitor
Media and general public	External	Public awareness and political pressure. AMA uses media to hold health ministers accountable	Low	Medium	Monitor

Figure 1: Stakeholder Register - Emergency Department Performance Crisis, Victoria and New South Wales (2024)

6.2 Power/Interest Matrix

The Power/Interest Matrix maps each stakeholder according to their level of power and interest, determining the appropriate engagement strategy for each group.

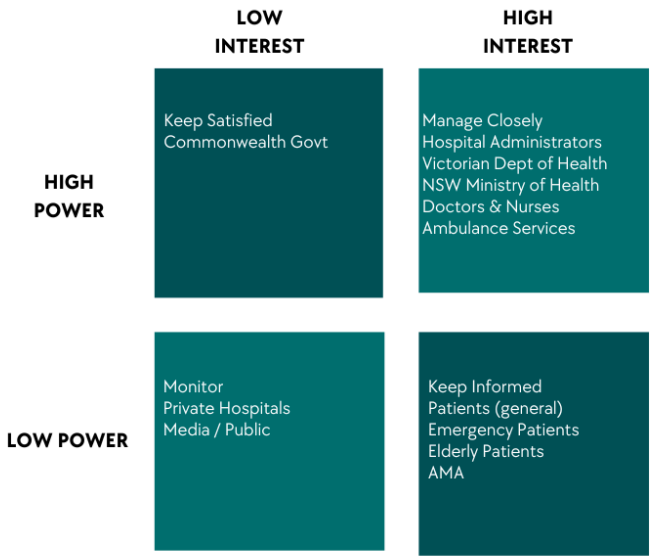

Figure 2: Power/Interest Matrix - Stakeholder Engagement Strategy

6.3 Key Insights

Three things stand out from this analysis. First, the most powerful stakeholders, namely governments and hospital executives, are not the ones most directly harmed. The people with the highest personal stake, patients, have the least power. This is a classic BA tension: the people who feel the problem most cannot fix it themselves.

Second, ambulance services are an often-overlooked stakeholder. The access block problem does not just affect patients inside the hospital. It creates a ripple effect that delays emergency responses across the whole community.

Third, the AMA itself is a key stakeholder in this analysis. They produced the report used as the primary source, which means they have a clear advocacy position. A good BA acknowledges where their source data comes from and whether the source has an agenda. In this case the AMA openly advocates for doctors and patients, which is worth noting.

7. Data Analysis

This analysis compares Emergency Department (ED) performance between Victoria and New South Wales (NSW) using real data from the Australian Institute of Health and Welfare (AIHW). Two key

measures are examined: the percentage of Category 3 urgent patients seen within 30 minutes, and the percentage of all ED patients completing their visit within 4 hours.

7.1 Key Finding 1: Category 3 Urgent Patients Seen Within 30 Minutes

In 2011-12 both NSW and Victoria had identical Category 3 Emergency Department (ED) urgent patient performance at 76%. By 2024-25 NSW maintained performance at 81% while Victoria declined significantly to 69%, a 12 percentage point gap between two comparable states. Victoria's sharpest decline occurred between 2017-18 and 2021-22, dropping from 73% to 62% before partially recovering.

AVERAGE of Percentage of patients seen on time	*State*		
Year	NSW	Vic	Grand Total
2011–12	76%	76%	76%
2012–13	79%	76%	78%
2013–14	83%	77%	81%
2014–15	84%	76%	82%
2015–16	85%	75%	83%
2016–17	85%	74%	83%
2017–18	85%	73%	82%
2018–19	83%	70%	80%
2019–20	84%	69%	80%
2020–21	84%	66%	79%
2021–22	83%	62%	77%
2022–23	82%	64%	77%
2023–24	82%	69%	79%
2024–25	81%	69%	77%
Grand Total	**83%**	**70%**	**80%**

Figure 3: Data Table - Average Percentage of Category 3 Patients Seen on Time, NSW vs Victoria (2011-12 to 2024-25)

Figure 4: Category 3 Emergency Department Urgent Patients Seen Within 30 Minutes - NSW vs Victoria (2011-12 to 2024-25)

7.2 Key Finding 2: 4-Hour Rule Performance

Analysis of the 4-hour rule, the percentage of all Emergency Department (ED) patients completing their visit within four hours, reveals a significant and widening performance gap between NSW and Victoria. In 2011-12 NSW recorded 73% and Victoria 69%, a difference of just 4 percentage points. By 2024-25 NSW had declined to 74% while Victoria fell sharply to 56%, a gap of 18 percentage points. Victoria's performance has now dropped below its 2011-12 starting point, suggesting systemic deterioration rather than a temporary setback.

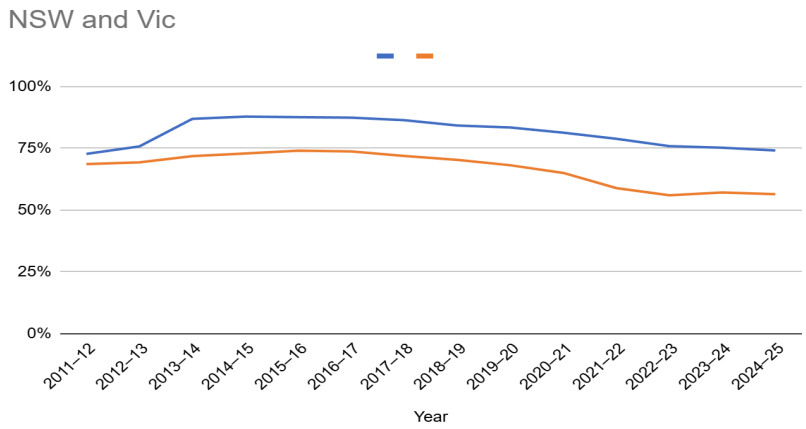

Figure 5: Percentage of All Emergency Department Patients Departing Within 4 Hours - NSW vs Victoria (2011-12 to 2024-25)

8. Power BI Dashboard

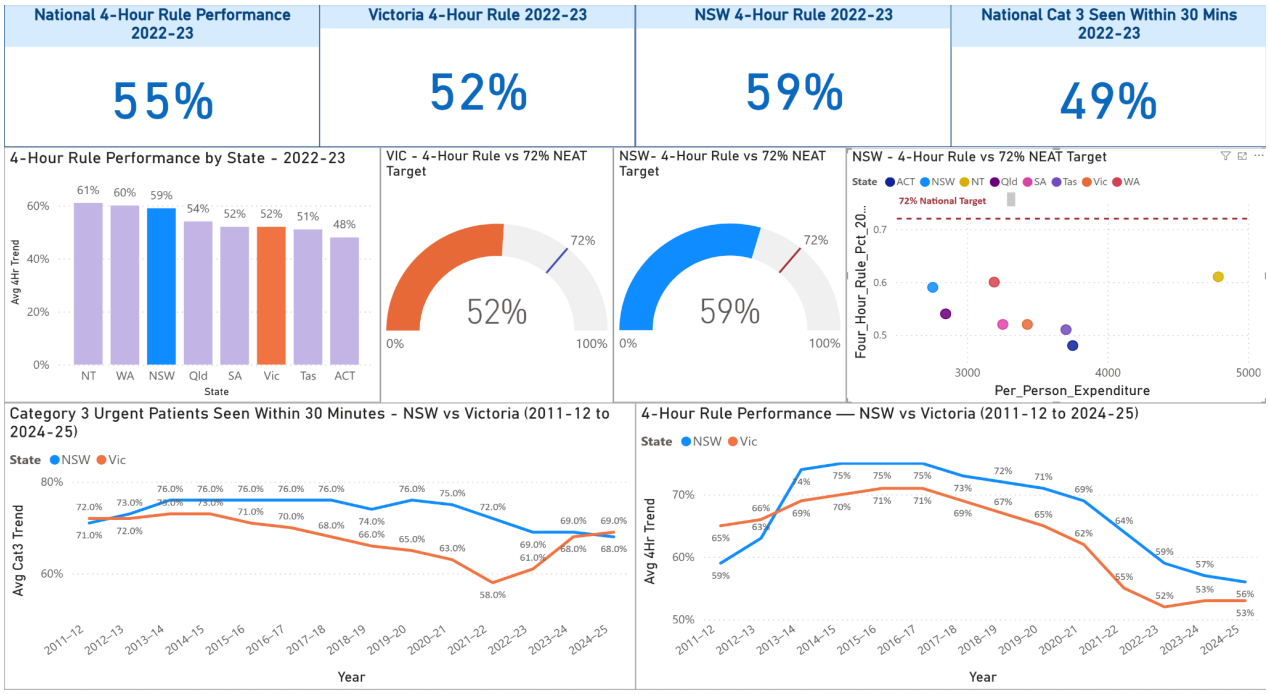

Figure 6: Power BI Dashboard - Australia's Public Hospital ED Performance: Victoria vs NSW, using AIHW Emergency Department Care Data (2011-12 to 2024-25)

The following interactive dashboard was developed in Microsoft Power BI using data from the Australian Institute of Health and Welfare (AIHW) Emergency Department Care dataset. It provides a single-page visual summary of the key performance metrics examined in this analysis, enabling at-a-glance comparison between Victoria and New South Wales across the 2022-23 financial year and across the full 2011-12 to 2024-25 trend period.

8.1 KPI Cards

The four KPI cards at the top of the dashboard (Figure 6) provide an immediate snapshot of national and state-level performance in 2022-23. The national 4-hour rule average of 55% confirms that barely half of all Australian ED patients are being processed within the clinically recommended timeframe. Victoria at 52% sits below the national average, while NSW at 59% outperforms it. The national Category 3 performance of 49%, meaning fewer than half of urgent patients were seen within 30 minutes, underscores the severity of the crisis at the triage level.

8.2 4-Hour Rule Performance by State

The bar chart in Figure 6 ranks all eight Australian states and territories by their 4-hour rule performance in 2022-23. NT leads at 61%, followed by WA at 60% and NSW at 59%. Victoria sits at 52%, below the national average of 55%. ACT records the lowest performance at 48%, meaning more than half of ACT emergency patients waited longer than four hours. No state approaches the 72% NEAT benchmark, confirming this is a national rather than isolated problem.

8.3 Gauge Charts: VIC and NSW vs 72% NEAT Target

The two gauge charts in Figure 6 directly visualise the gap between actual state performance and the National Emergency Access Target (NEAT) of 72%. The NEAT benchmark requires that 72% of all ED patients be seen, treated, and either admitted, discharged, or transferred within four hours of arrival. Victoria's needle sits at 52% against this benchmark, a shortfall of 20 percentage points. NSW performs better at 59% but still falls 13 percentage points short. Both gauges make the underperformance immediately visible without requiring the reader to interpret raw numbers.

8.4 Scatter Plot: Spending vs ED Performance by State

The scatter plot in Figure 6 examines whether higher per-person expenditure correlates with better 4-hour rule performance across states. The dashed red reference line at 0.72 represents the NEAT benchmark and no state reaches it. Notably, NT records both the highest expenditure at approximately $4,900 per person and the strongest performance at 62%, however this relationship does not hold across other states. ACT spends approximately $3,600 per person yet records the weakest performance at 47%. NSW achieves relatively strong performance at 59% despite spending only approximately $2,700 per person, suggesting that efficiency rather than volume of funding is the more important driver. The absence of a clear linear trend supports the central argument of this report: increased funding alone does not guarantee improved outcomes.

8.5 Line Charts: NSW vs Victoria Trend (2011-12 to 2024-25)

The two line charts at the bottom of Figure 6 illustrate the long-term performance trajectories of NSW and Victoria across both key metrics. For Category 3 urgent patients seen within 30 minutes, both states show a declining trend, however Victoria's deterioration is notably steeper, dropping from 76% in 2011-12 to 69% in 2024-25, while NSW maintained stronger performance. For the 4-hour rule, the divergence is even more pronounced. NSW declined from a peak of 75% in 2016-17 to 57% in 2024-25, while Victoria fell from 75% to just 53% over the same period. Victoria's performance has consistently sat below NSW since approximately 2017-18 and the gap has widened each year since, suggesting a structural rather than temporary decline.

9. AS-IS Process Map

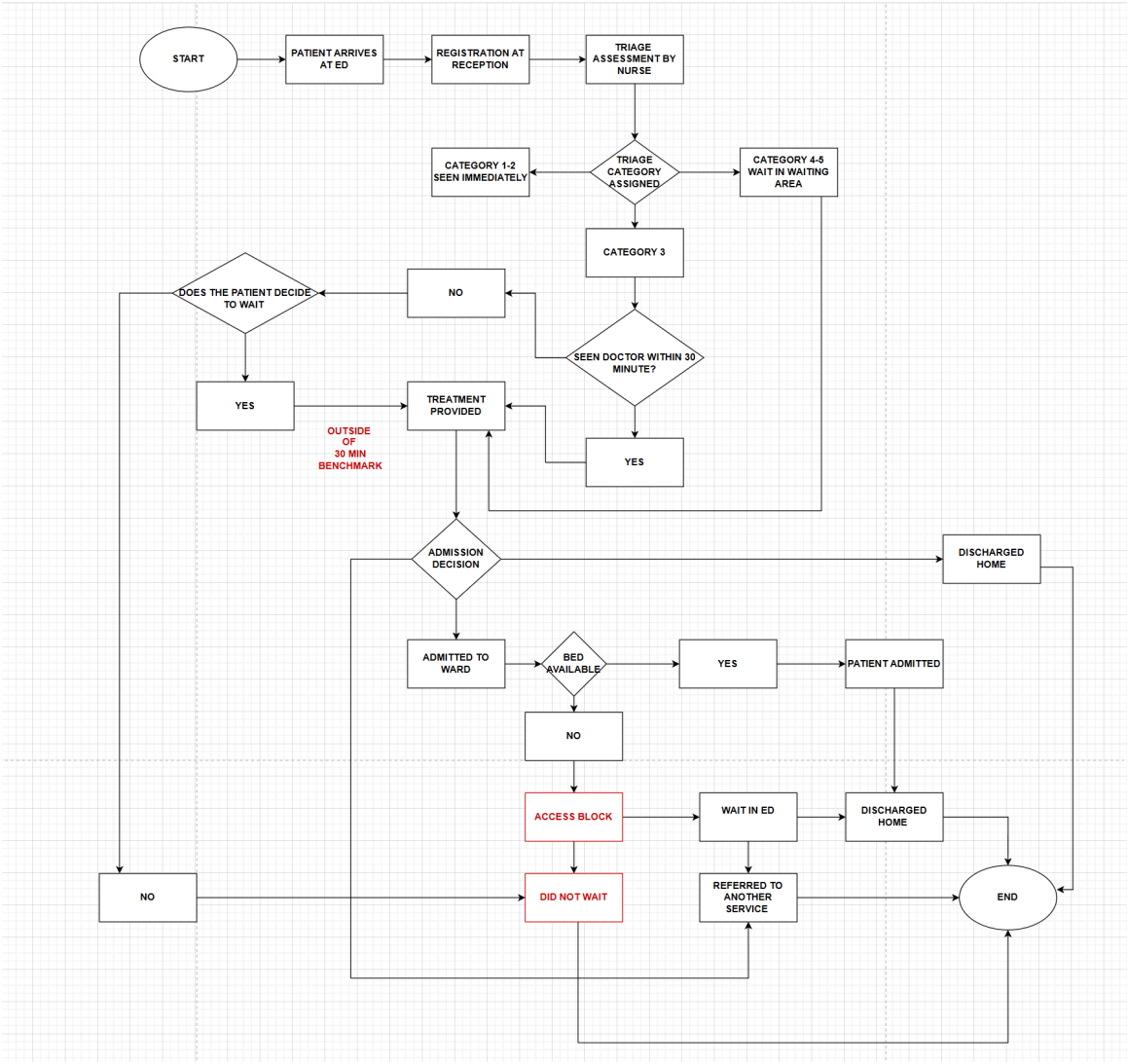

Figure 7: AS-IS Process Map - Emergency Department Patient Journey (Current State, 2024) based on process analysis informed by the AMA 2024 Public Hospital Report Card

For Category 3 urgent patients, the process reveals two critical failure points. First, if a patient is not seen by a doctor within 30 minutes, they face a choice: continue waiting or leave without being treated. Patients who leave are recorded as Did Not Wait (DNW) events, representing a complete failure of the system to deliver care. Patients who choose to continue waiting will eventually receive treatment, but outside the recommended timeframe, meaning the 30-minute benchmark has already been missed.

Second, after treatment is provided, a patient requiring hospital admission faces another potential failure point: Access Block. This occurs when no ward bed is available, forcing the patient to remain in the ED while waiting for a bed to become available. This is identified by the AMA as one of the primary drivers of ED wait time blowouts, as it prevents new patients from being seen and processed efficiently.

The two red boxes in the diagram, Access Block and Did Not Wait, represent the critical failure points in the current process. These are the areas where the system breaks down and patient outcomes are most severely impacted.

10. Root Cause Analysis

The Fishbone diagram below identifies the root causes contributing to declining Emergency Department (ED) performance across Victoria and New South Wales (NSW). Six cause categories were identified through analysis of the AMA 2024 Public Hospital Report Card and AIHW data.

Demand pressures are driven by rapid population growth and an ageing population. Australians over 65 represent 47% of patient days despite being only 17% of the population. Workforce shortages reflect chronic understaffing, burnout, and difficulty retaining staff in the public system. Capacity constraints include critically low bed ratios and access block. Funding is increasing but not translating into performance improvement. Process failures such as inefficient patient flow and fragmented handoffs create internal bottlenecks. Finally, policy gaps, particularly in aged care and primary care, are pushing patients into emergency departments unnecessarily.

These causes do not operate in isolation. They are interconnected: policy gaps drive demand, which overwhelms capacity, which worsens process flow, which burns out the workforce. This interconnected nature means that addressing only one cause in isolation will not resolve the problem.

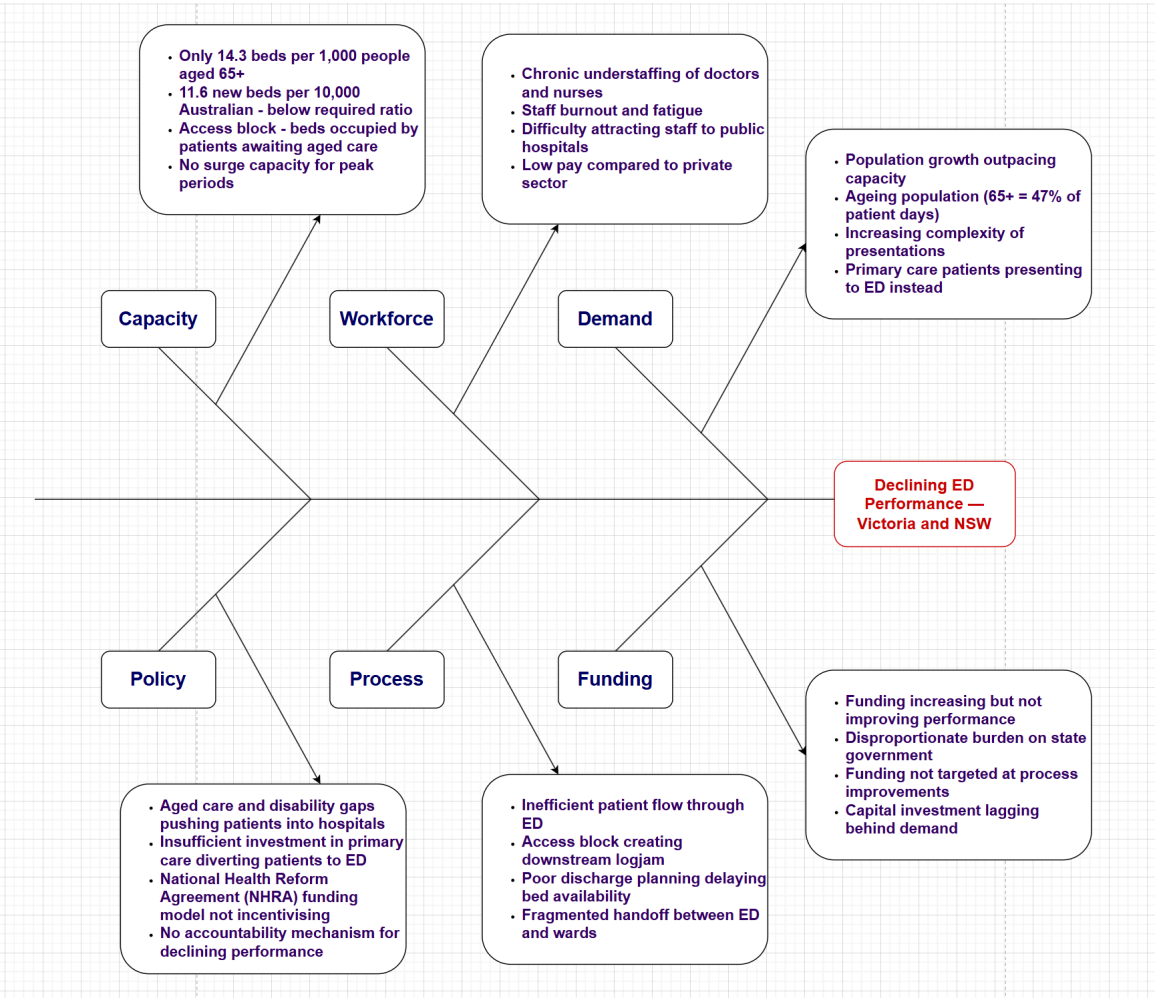

Figure 8: Root Cause Analysis - Fishbone Diagram based on the AMA 2024 Public Hospital Report Card.

11. TO-BE Process Map

The TO-BE process map illustrates the improved Emergency Department (ED) patient journey following the implementation of the recommended changes. When compared to the AS-IS map, the future state process is notably simpler, reflecting fewer failure points and a more efficient patient flow.

The most significant structural change occurs before the patient even arrives at the ED. A Pre-ED Clinical Triage Assessment step has been introduced, where a qualified clinician assesses the patient's condition remotely and determines whether ED attendance is necessary. This may be conducted via telephone triage services, digital symptom checkers, or ambulance pre-triage.

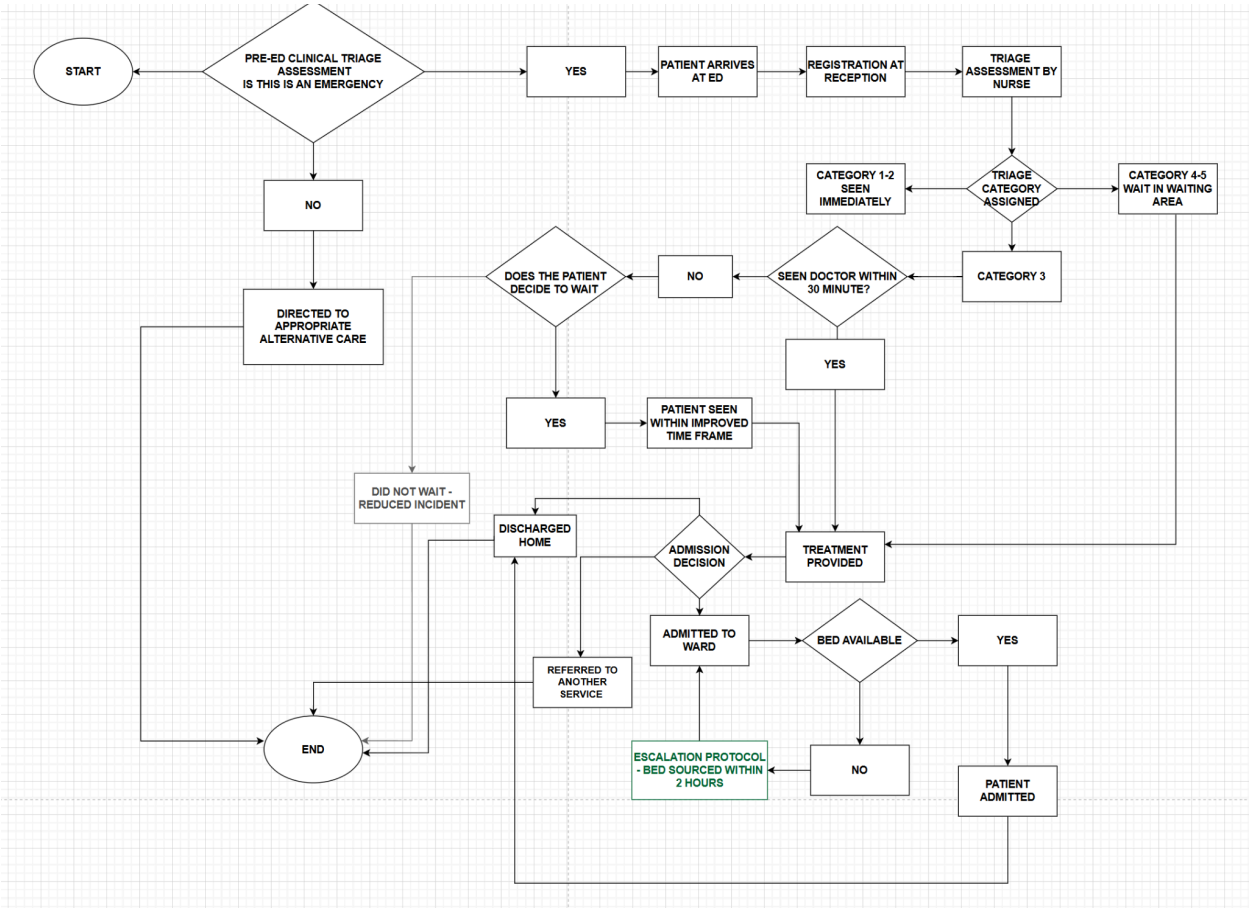

Figure 9: TO-BE Process Map - Improved Emergency Department Patient Journey (Future State) based on recommendations informed by the AMA 2024 Public Hospital Report Card

Patients assessed as non-emergency are directed to appropriate alternative care such as an urgent care clinic, General Practitioner (GP) service, or telehealth consultation, reducing unnecessary ED presentations and freeing capacity for genuine emergencies.

For patients who do require ED care, the internal process follows the same triage and assessment structure as the current state. However two critical improvements are reflected in the future state. First, Category 3 urgent patients who are not seen within 30 minutes now have access to care within an improved timeframe due to increased staffing levels, reducing both wait times and Did Not Wait (DNW) events. DNW events are still possible in the TO-BE state as patients retain the right to leave, however they are now an exception rather than a systemic failure, reflected by the grey colouring of the DNW box compared to red in the AS-IS map.

Second, the Access Block failure point has been completely eliminated from the future state process. Where the AS-IS map showed patients becoming stranded in the ED due to unavailable ward beds, the TO-BE map introduces an Escalation Protocol where a bed is actively sourced within two hours,

ensuring admitted patients move out of the ED promptly and maintaining patient flow throughout the department.

The combined effect of these improvements is a significantly reduced burden on the ED, faster patient throughput, lower DNW rates, and the elimination of access block as a systemic failure.

12. Recommendations and RACI Matrix

12.1 Recommendations

Six recommendations are proposed in priority order:

RECOMMENDATION	RESPONSIBLE	TIMEFRAME
Introduce Pre-ED Clinical Triage Assessment	VIC & NSW Health Departments	6 to 12 months
Expand hospital bed and room capacity	Commonwealth & State Governments	12 to 24 months
Increase workforce through targeted recruitment	VIC & NSW Health Departments	12 to 18 months
Invest in staff training for aged care presentations	Hospital Administrators	6 to 12 months
Implement Escalation Protocol for bed sourcing within 2 hours	Hospital Administrators	3 to 6 months
Expand after-hours GP and urgent care clinic availability	Commonwealth Government & GPs	12 to 24 months

Figure 10: Recommendations - Priority Order, Responsible Parties and Implementation Timeframes

12.2 RACI Matrix

The RACI Matrix above outlines the roles and responsibilities of each key stakeholder in implementing the six recommendations identified in this analysis. The matrix uses four classifications: Responsible, Accountable, Consulted, and Informed, to clearly define who does the work, who owns the outcome, who provides input, and who is kept updated.

The matrix reveals two clear patterns. First, the Commonwealth Government holds accountability for the two largest structural recommendations, bed capacity expansion and after-hours GP services, reflecting the scale of investment required at the federal level. Second, Hospital Administrators and Clinical Staff hold primary responsibility for the two operational recommendations, staff training and the escalation protocol, as these are process-level changes implemented at the hospital level rather than requiring government intervention.

This distinction between government-level and hospital-level responsibilities is important because it means some recommendations can be implemented immediately without waiting for policy or funding changes, while others require longer term government commitment.

Recommendation	Commonwealth Govt	VIC Dept of Health	NSW Ministry of Health	Hospital Administrators	Clinical Staff	Primary Care / GPs
1. Pre-ED Clinical Triage Assessment	Accountable	Responsible	Responsible	Consulted	Consulted	Informed
2. Expand Hospital Bed Capacity	Accountable	Responsible	Responsible	Consulted	Informed	Informed
3. Increase Workforce	Consulted	Responsible	Responsible	Accountable	Consulted	Informed
4. Staff Training - Aged Care	Informed	Consulted	Consulted	Responsible	Accountable	Informed
5. Escalation Protocol - Bed Sourcing	Informed	Consulted	Consulted	Responsible	Accountable	Informed
6. Expand After-Hours GP Services	Accountable	Consulted	Consulted	Informed	Informed	Responsible

Figure 11: RACI Matrix - Recommendations Implementation based on stakeholder analysis and recommendations informed by the AMA 2024 Public Hospital Report Card

13. Expected Outcomes

If these recommendations are implemented effectively, the following outcomes are projected: reduction in unnecessary ED presentations through pre-ED triage and expanded GP availability; elimination of access block as a systemic failure through bed capacity expansion and the escalation protocol; faster patient throughput and reduced Did Not Wait (DNW) events through improved staffing and training; improved performance against both the 30-minute Category 3 benchmark and the 4-hour rule; and a more sustainable and resilient public hospital system capable of meeting the demands of a growing and ageing population.

14. Conclusion

The data is clear. Australia's public hospital system is under significant and growing strain, and Victoria in particular is experiencing a performance crisis that requires urgent attention. The solution is not simply more funding. It is smarter, more targeted investment combined with genuine process-level reform. The recommendations in this report provide a practical, evidence-based roadmap for improvement that can be implemented across both state and federal levels of government.

15. References

Australian Medical Association (AMA) (2024). 2024 Public Hospital Report Card. AMA, Barton ACT. Published April 2024. Retrieved from https://www.ama.com.au

Note: All data cited in this analysis is drawn directly from the AMA 2024 Public Hospital Report Card, which itself sources data from the Australian Institute of Health and Welfare (AIHW) and the Australian Bureau of Statistics (ABS). Page references are provided inline throughout this document.